

Ponatinib / Asciminib nella LMC – Newsletter interna (ultimi 15 giorni)

Generata il 01/07/2026

Generata il: 2026-07-01

Newsletter interna – sintesi operativa

Messaggi chiave (da leggere in 60 secondi):

- Nel periodo emergono soprattutto temi su **TFR / stop terapia** (n=4).
- Secondo filone: **Sequencing / linee di terapia** (n=2).
- Terzo filone: **Sintesi (review/linee guida)** (n=2).

Cosa cambia per noi (azioni pratiche):

- Valutare criteri interni per **consolidamento / de-escalation / TFR** nei pazienti in deep response.
- Verificare percorso di **test mutazionale** e criteri di switch nelle resistenze (incl. T315I).

Safety radar (segnali/interazioni da monitorare):

- Monitorare evoluzione su **TFR / stop terapia** (nuovi segnali/dati).
- Monitorare evoluzione su **Resistenza / T315I / mutazioni** (nuovi segnali/dati).

Top 5 da leggere subito (ordinati per impatto)

1) Molecular Relapse After a Second Treatment-Free Remission Attempt Following Asciminib in Chronic Myeloid Leukemia: A Case Report.

Linee guida / consenso

Case reports in hematology

Score: 19

Focus clinico: TFR / stop terapia, Dose strategy / consolidamento, Sequencing / linee di terapia, Sintesi (review/linee guida)

PMID: 42369861 | 10.1155/crh/1621284

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42369861/>

- **Tipo di studio/popolazione:** In this study, we report the case of a man in his 60s with chronic-phase CML who experienced molecular relapse 8 months after a second TFR attempt following asciminib treatment.
- **Risultato chiave:** The patient was diagnosed with CML approximately 15 years earlier and achieved deep molecular response 4.5 (MR4.5), representing a 4.5-log reduction in *BCR::ABL1* transcripts, with first-line imatinib.
- **Rilevanza clinica:** Asciminib was subsequently introduced as third-line therapy at a dose of 40 mg twice daily, and *BCR::ABL1* transcripts remained undetectable for more than 3 years.

2) Four Decades of Molecular Innovation in Chronic Myeloid Leukemia: From Antisense Targeting to Treatment-Free Remission.

Linee guida / consenso

Cancers

Score: 17

Focus clinico: Resistenza / T315I / mutazioni, TFR / stop terapia, Sintesi (review/linee guida)

PMID: 42352456 | 10.3390/cancers18121922

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42352456/>

- **Tipo di studio/popolazione:** Chronic myeloid leukemia (CML) represents a paradigm of targeted therapy, driven by the BCR::ABL1 fusion kinase.
- **Risultato chiave:** Over the past four decades, therapeutic strategies have evolved from early molecular targeting approaches and interferon- α to tyrosine kinase inhibitors (TKIs), dramatically improving survival and transforming CML into a largely controllable disease.
- **Rilevanza clinica:** Furthermore, the achievement of deep molecular responses has enabled TFR in approximately 40-60% of selected patients, redefining treatment goals toward functional cure.

3) Asciminib Provides Better Efficacy and Favorable Safety and Tolerability Against Investigator-Selected Tyrosine Kinase Inhibitors in East Asian Patients With Newly Diagnosed Chronic Myeloid Leukemia: Results From a Subgroup Analysis of the Pivotal ASC4FIRST Study.

Trial clinico

Cancer medicine

Score: 7

Focus clinico: TFR / stop terapia, Sequencing / linee di terapia

PMID: 42339511 | 10.1002/cam4.72019

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42339511/>

- **Tipo di studio/popolazione:** ASC4FIRST (ClinicalTrials.gov NCT04971226) is a pivotal phase 3 clinical trial comparing the efficacy and safety of asciminib vs.
- **Risultato chiave:** Asciminib also showed a favorable safety profile in East Asian patients, with lower rates of Grade ≥ 3 adverse events (AEs) and AEs leading to discontinuation compared to IS-TKIs (41.9% vs.
- **Rilevanza clinica:** 9.5%, respectively), consistent with the results from non-East Asian patients.

4) Independent PML::RARA-positive acute promyelocytic leukemia arising during BCR::ABL1-positive chronic myeloid leukemia.

Altro

Journal of clinical and experimental
hematopathology : JCEH

Score: 4

Focus clinico: TFR / stop terapia

PMID: 42366058 | 10.3960/jslrt.26010

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42366058/>

- **Tipo di studio/popolazione:** We report an octogenarian man with chronic-phase CML who was intolerant to multiple tyrosine kinase inhibitors and underwent temporary discontinuation of asciminib.
- **Risultato chiave:** During molecular relapse, bone marrow examination revealed 88% abnormal promyelocytes with Auer rods and markedly elevated PML::RARA transcripts, whereas BCR::ABL1 transcript levels were more than 3 logs lower than at CML diagnosis.
- **Rilevanza clinica:** Conventional cytogenetics demonstrated t(15;17) without t(9;22) as the dominant clone, supporting de novo APL arising independently of the residual CML clone.

5) Dual functional genomics reveals a broad and convergent landscape of asciminib resistance in BCR::ABL1.

Preclinico / laboratorio

Genome medicine

Score: 2

Focus clinico: Resistenza / T315I / mutazioni

PMID: 42365381 | 10.1186/s13073-026-01679-x

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42365381/>

- **Tipo di studio/popolazione:** The allosteric inhibitor asciminib is a novel therapy for chronic myelogenous leukemia (CML) that targets the myristoyl pocket of the BCR::ABL1 kinase.
- **Risultato chiave:** This study employed a dual functional genomics approach in CML cell line models.
- **Rilevanza clinica:** The screens identified 279 asciminib resistance mutations and revealed resistance hotspots distributed across the SH3, SH2, and kinase domains, in contrast to imatinib resistance, which is largely confined to the kinase domain.

Evidenze per domande cliniche

TFR / stop terapia (4)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Molecular Relapse After a Second Treatment-Free Remission Attempt Following Asciminib in Chronic Myeloid Leukemia: A Case Report.

Linee guida / consenso

Case reports in hematology

Score: 19

PMID: 42369861 | 10.1155/crh/1621284

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Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** In this study, we report the case of a man in his 60s with chronic-phase CML who experienced molecular relapse 8 months after a second TFR attempt following asciminib treatment.
- **Risultato chiave:** The patient was diagnosed with CML approximately 15 years earlier and achieved deep molecular response 4.5 (MR4.5), representing a 4.5-log reduction in *BCR::ABL1* transcripts, with first-line imatinib.
- **Rilevanza clinica:** Asciminib was subsequently introduced as third-line therapy at a dose of 40 mg twice daily, and *BCR::ABL1* transcripts remained undetectable for more than 3 years.

Mini-riassunto (da abstract, estrattivo):

Four Decades of Molecular Innovation in Chronic Myeloid Leukemia: From Antisense Targeting to Treatment-Free Remission.

Linee guida / consenso

Cancers

Score: 17

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- **Tipo di studio/popolazione:** Chronic myeloid leukemia (CML) represents a paradigm of targeted therapy, driven by the *BCR::ABL1* fusion kinase.
- **Risultato chiave:** Over the past four decades, therapeutic strategies have evolved from early molecular targeting approaches and interferon- α to tyrosine kinase inhibitors (TKIs), dramatically improving survival and transforming CML into a largely controllable disease.
- **Rilevanza clinica:** Furthermore, the achievement of deep molecular responses has enabled TFR in approximately 40-60% of selected patients, redefining treatment goals toward functional cure.

Mini-riassunto (da abstract, estrattivo):

Riassunto aggiuntivo (full text Open Access, estrattivo):

Asciminib Provides Better Efficacy and Favorable Safety and Tolerability Against Investigator-Selected Tyrosine Kinase Inhibitors in East Asian Patients With Newly Diagnosed Chronic Myeloid Leukemia: Results From a Subgroup Analysis of the Pivotal ASC4FIRST Study.

Trial clinico

Cancer medicine

Score: 7

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Open Access (Europe PMC/PMC): sì (PMC13291802)

- **Tipo di studio/popolazione:** ASC4FIRST (ClinicalTrials.gov NCT04971226) is a pivotal phase 3 clinical trial comparing the efficacy and safety of asciminib vs.
- **Risultato chiave:** Asciminib also showed a favorable safety profile in East Asian patients, with lower rates of Grade ≥ 3 adverse events (AEs) and AEs leading to discontinuation compared to IS-TKIs (41.9% vs.
- **Rilevanza clinica:** 9.5%, respectively), consistent with the results from non-East Asian patients.
- **Metodi (full text OA):** Briefly, eligible patients were aged ≥ 18 years and had Philadelphia chromosome-positive (Ph+) CML-CP as per the European LeukemiaNet (ELN) 2020 criteria [29], which had been newly diagnosed within 3 months of study enrollment.
- **Risultato principale (full text OA):** Notably, more East Asian patients also achieved deep molecular response (DMR) with asciminib than all IS-TKIs (Week 48: MR4.0, 40.5% vs.
- **Dettaglio utile (full text OA):** Deep responses at Week 48 and Week 96 in East Asian and non-East Asian patients in the asciminib and IS-TKI arms (top row), asciminib vs.
- **Messaggio clinico (full text OA):** This suggests similar levels of asciminib exposure, in contrast with results from a previous study showing that Chinese patients with CML had higher imatinib Ctrough values than Western patients [28].

Mini-riassunto (da abstract, estrattivo):

Independent PML::RARA-positive acute promyelocytic leukemia arising during BCR::ABL1-positive chronic myeloid leukemia.

Altro

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- **Rilevanza clinica:** Conventional cytogenetics demonstrated t(15;17) without t(9;22) as the dominant clone, supporting de novo APL arising independently of the residual CML clone.

Sequencing / linee di terapia (2)

Score: 0

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Linee guida / consenso

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Sintesi (review/linee guida) (2)

Score: 0

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Resistenza / T315I / mutazioni (2)

Score: 0

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Genome medicine

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Dose strategy / consolidamento (1)

Score: 0

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